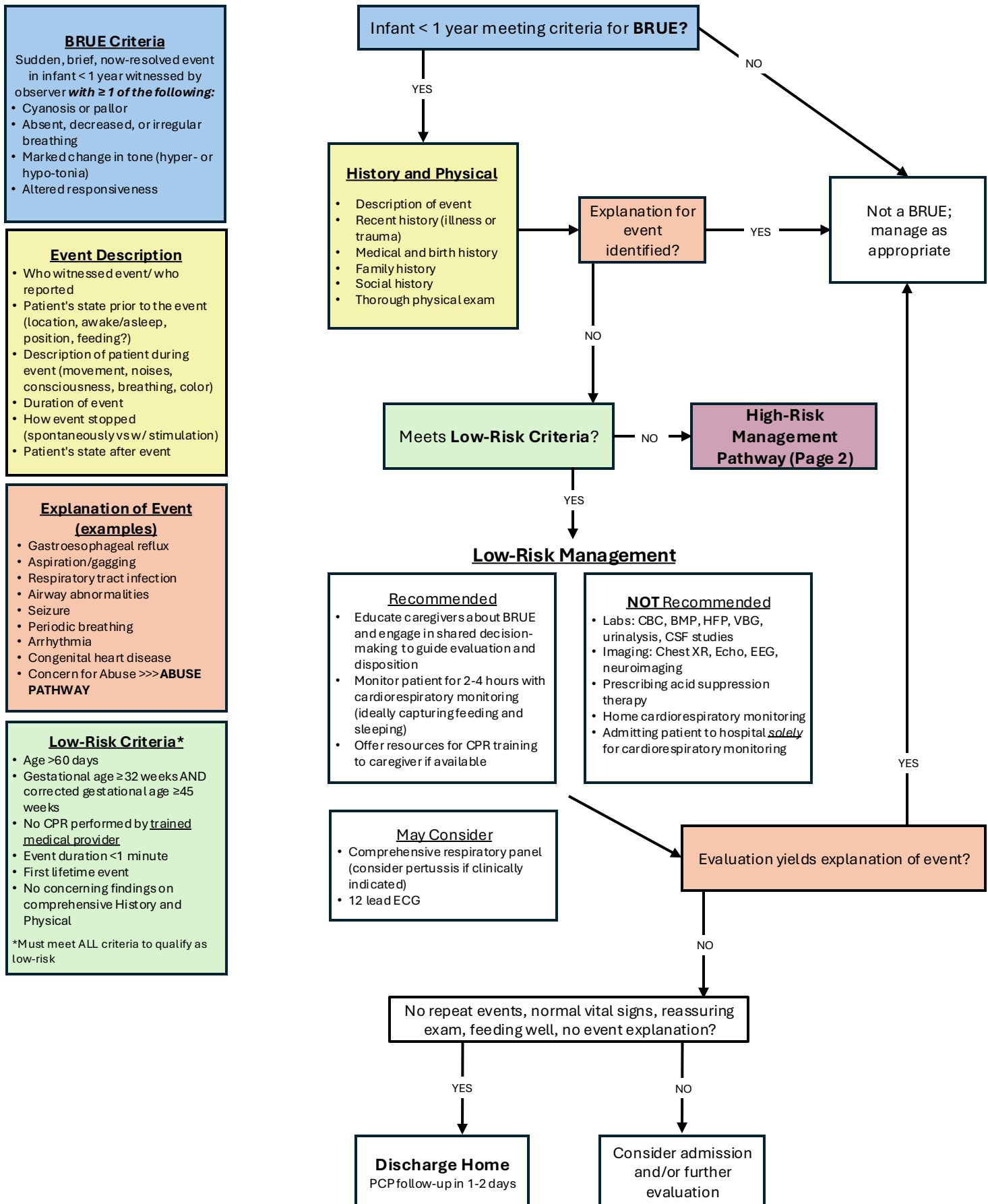


BRUE (Brief Resolve Unexplained Event) Management Pathway

Page 1 – Initial Assessment and Low-Risk Management



BRUE (Brief Resolve Unexplained Event) Management Pathway

Page 2 – High-Risk Management

High Risk Criteria*

- Age <60 days
- Gestational age <32 weeks
- Gestational age ≥ 32 weeks with corrected gestational age <45 weeks
- CPR performed by trained medical provider
- Event lasted >1 minute
- Previous BRUE (regardless of time interval)
- Concerns identified during History and Physical

*Patient qualifies if they meet ANY criterion

Explanation of Event (examples)

- Gastroesophageal reflux
- Aspiration/gagging
- Respiratory tract infection
- Airway abnormalities
- Seizure
- Periodic breathing
- Arrhythmia
- Congenital heart disease
- Concern for Abuse >>>**ABUSE PATHWAY**

Patient Satisfies Criteria for High-Risk BRUE

Emergency Department Management

- Start continuous pulse oximetry monitoring
- **May consider**, per patient factors:
 - Comprehensive respiratory panel
 - Pertussis PCR testing
 - 12 lead ECG
 - CBC, CMP, VBG

Evaluation yields explanation of event?

YES

Not a BRUE; manage as appropriate

NO

Admit to Pediatric Hospital Medicine

- Prolonged cardiorespiratory monitoring (ideally capturing sleep and feeding)
- Observe for repeat events
- Strongly consider speech therapy consultation for feeding/swallow evaluation
- May consider additional targeted evaluation for a non-BRUE diagnosis, based on event characteristics and/or recurrence

Evaluation yields explanation of event?

YES

NO

Consider discharge with diagnosis of BRUE after prolonged observation **if all the following are met:**

- There are no ongoing events
- Vital signs are normal
- Exam is reassuring
- Patient is feeding well
- No explanation is identified

Discharge Home

- Provide CPR teaching/resources for parents, as available (e.g. AHA Infant CPR Training Kit)
- Ensure close follow-up with PCP in 1-2 days